

Anaemia-Free Futures: Improving Adolescent Nutrition in Tribal Districts

Program Context

Adolescent girls in tribal blocks of eastern India face high levels of iron-deficiency anaemia, with limited access to effective nutrition education or health supplements. Despite ongoing IFA tablet distribution, uptake remains low due to stigma, lack of knowledge, and poor monitoring.

Theory of Change Pathway

If adolescent girls receive community-based nutrition education and regular school engagement, and frontline workers are equipped with interactive tools to track IFA intake, then knowledge and supplement uptake will improve, leading to reductions in anaemia prevalence among this group.

Target Group & Geography

Target: Girls aged 10-19 years in tribal residential schools and anganwadi centres.

Location: 4 tribal-dominated blocks in [State], India.

Anaemia-Free Futures: Improving Adolescent Nutrition in Tribal Districts

Outcomes

Short-term: Increased knowledge of anaemia and IFA benefits.

Medium-term: Improved adherence to IFA consumption.

Long-term: Reduced prevalence of moderate to severe anaemia in adolescent girls.

Key Assumptions

- Girls attend sessions regularly
- Schools support health engagement
- Tablets are available and frontline workers are not overburdened

Monitoring Approach

Routine school tracking forms, periodic surveys on knowledge and compliance, and quarterly review meetings with anganwadi and ASHA workers to assess gaps and corrective actions.

Note

This Theory of Change is a working model to guide program design, MEL strategy, and stakeholder alignment. It should be revisited periodically as the context evolves.